جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه عبدالسلام جوده حسانين
المحافظة : الجيزه **الحي / المدينة :** ابوالنمرس **العنوان :** ابو النمرس
رقم البطاقة : 26902172200763 **رقم التليفون :** 01555148881 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 17-02-1969 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و بيرتوزوماب (فيسجو) [14117] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه عبدالسلام جوده حسانين
National ID: 26902172200763

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Phesgo) **Subcutaneous trastuzumab and pertuzumab)**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

الهيئة العامة للتامين الصحي
الادارة المركزية لنظم المعلومات و التحول الرقمي

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION SYSTEM AND DIGITAL TRANSFORMATION

ربيقرم الاستعلام 20 جنية

تاريخ الاصدار :5-7-2027

جهه الاصدار بفرع الجيزة

الى من يهمه الامر .....

الهيئه العامة للتامين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التامين الاجتماعي رقم 79 لسنه 1975 المعدل بالقانون 25 لسنه 1977 "القوى العامله والمعاشات والازامل " و كذلك قانون 99 لسنه 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنه 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنه 2012 الخاص بالمرأة المعيله .

وطبقا لقاعدة البيانات المرسلة من التامينات الاجتماعيه

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي

والبحث تبين أن

السيد (ة) فاطمه عبدالسلام جوده حسنين بطاقة رقم 26902172200763

مقيدون رقم : لا يوجد

وطبقا للى خطاب من التامينات الاجتماعية يفيد عدم الانتفاع

يتبين بان المذكور غير المنتفعت بالتامين الصحي صدر هذا البنطاب دون ذني مسئولية على الهيئة العامة للتامين الصحي حال خضوع المواطن لقانون التامين الصحي الشامل

وشكرا

تاريخ الطباعة :5-7-2026

الموظف المختص

احمد ياسر

يأكيد و فن نظم المعلومات

يتعمد:::

مسلسل 1777536

للطالب سيران مدة ثلاثة اشهر من تاريخ الاصدار الا في حاله تغيير حالته المواطن من قبل الهيئة الفرعية للتامينات الاجتماعية أو وزارة التضامن الاجتماعي أو وزارة التربية و التعليم

الادارة المركزية لنظم المعلومات و التحول الرقمي

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name: فاطمه عبدالسلام جوده حسانين	
MR. : 3166	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 56-year-old individual with a history of right breast cancer, staged T3 N1, who has undergone neoadjuvant chemotherapy (NACT) with a taxol–carboplatin backbone followed by a phesgo (paclitaxel 600 mg SC) regimen. Initial MDT discussion on 1 December 2025 noted a good clinical response with no palpable mass and a T2 N1 status. The patient received taxol 130 mg and carboplatin AUC 2 during week 9, followed by taxol 130 mg and carboplatin AUC 2 in week 10, and then taxol 130 mg with carboplatin AUC 2 plus phesgo 600 mg SC in week 11. Subsequent cycles continued with taxol 130 mg and carboplatin AUC 2 through week 13, and phesgo was administered in cycle 4 (week 14) with taxol 130 mg and carboplatin AUC 2. In week 15 the patient received taxol 130 mg and carboplatin AUC 2 (200 mg) with labs accepted on 7 March 2026; the same regimen was repeated in week 16 with labs accepted on 21 March 2026. Week 17 included taxol 130 mg, carboplatin AUC 2, and phesgo C5, with a new CBC on 28 March 2026 showing neutropenia. The patient's left ventricular ejection fraction remained stable at 65 % during this period.

Post-NACT, the patient underwent a stereotactic radiosurgery (SRS) procedure with complete response noted on 5 April 2026. Subsequent management included a port insertion and initiation of adjuvant endocrine therapy with aromatase inhibitor (AI) and continued phesgo. On 6 June 2026, the patient received cycle 7 of phesgo 600 mg SC, femara, and supportive therapy, with an echocardiogram on 4–5 June 2026 showing EF 67 %. Cycle 8 of phesgo was administered on 12 July 2026 with continued femara and bone-protective agents (calcemate and bone care). On 6 May 2026, the patient received cycle 9 of phesgo and arimidex, with new labs and a venous duplex performed. Cycle 10 of phesgo and arimidex was given on 16 August 2026, with a venous duplex showing free flow and EF 64 %. The patient's port was removed on 8 September 2026, after which cycle 11 of phesgo and arimidex was administered, followed by cycle 12 of phesgo and arimidex.

The patient has completed 12 cycles of phesgo.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

USTAVE / OUSSY
ERNATIONAL

Patient Name	: الأستاذه / فاطمه عبدالسلام جوده	Request No.	: 13006324
Patient No.	: 26902172200763	Request Date	: 06/05/2026
Path No.	: 1341/26	Verification Date	: 17/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 57 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.
Nature of Specimen: Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes biopsy.

Gross Examination:
* Right breast wide local excision specimen measured 08x03x03 cm, with covering skin ellipse measured 03x02 cm.
* Serialling showed a wire guided tumor bed site measured 2x2x1 cm, located 0.3 cm from the least inferior margin.
* Sentinel lymph nodes: Fibrofatty tissue piece measured 4x3x1 cm, entangling 6 lymph nodes, the largest measured 1.5x1x0.5 cm.
* Extra lymph nodes: Fibrofatty tissue piece entangling 2 lymph nodes, each measured 1x1x0.5 cm.
* Total lymph nodes; Eight lymph nodes.
* 12 Blocks:
1- Medial margin., 2- Lateral margin., 3- Superior margin., 4- Inferior margin., 5- Deep margin., 6- SLNs., 7:10- Wire guided area., 11&12- Extra SLNs.
- Gross was done by; Dr. Heba El-Shafeey.

** Frozen (Intra-operative) results:
- Surgical margins: Free.
- SLNs: Six free sentinel lymph nodes (0/6).

Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE / OUSSY
ERNATIONAL

Patient Name	: الأستاذه / فاطمه عبدالسلام جوده	Request No.	: 13006324
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Age	: 57 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

* Other findings: Extensive therapy effect.
* Surgical margins: Free.
* Sentinel lymph nodes: Six free sentinel lymph nodes, SLNs (0/6).
* Extra lymph nodes: Two free lymph nodes (0/2).
* Total nodal status: Eight free lymph nodes (0/8).
* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Right breast cancer (Post-NACT), Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes:
- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Surgical margins: Free.
* Total nodal status: Eight free lymph nodes (0/8).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

06/01/2026

MDT FORM

Name:	Fatma Abdelaziz Gouda Hassanin	MRN:	3166
Age:	57 Y	ID:	26902170200763
Menopaus: Status:	post	Phone:	01003765514
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	-VE	Reason Of MDT:	post op

Clinical Data:

BX: RIGHT IDC G2

ER 9 PR 7 HER 3 K167 50

R L



T N M

Sonomammography:

Mets Workup:

History:

Initially ct2 n1Right breast cancer (Post-NACT), Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes: - No residual viable remnants of the previously diagnosed invasive tumor. - Ductal carcinoma in situ: Absent. - Surgical margins: Free. - Total nodal status: Right free lymph nodes (0/3). - Histologic response to therapy:

Recommendations:

for adj rtt

MDT Outcome & Plan

POST
CONTINUE ADJUVANT PHESGO
ADJUVANT AI

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

06/01/2026

MDT FORM

Name:	Fatma Abdelsalam Gouda Hassanin	MRN:	3166
Age:	57 Y	ID:	29902172200763
Menopausal Sta	post	Phone:	01005766814
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	-VE	Reason Of MDT:	post op

Clinical Data:

BX: RIGHT IDC G2

ER: 0 PR: 7 HER2: 3 KI67: 50

Sonomammography:

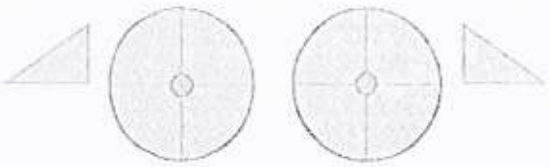
Mets Workup:

History:
Initially ct2 n1Right breast cancer (Post-NACT). Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes: - No residual viable remnants of the previously diagnosed invasive tumor. ? Ductal carcinoma in situ: Absent. ? Surgical margins: Free. * Total nodal status: Eight free lymph nodes (0/8). - Histologic response to therapy: Pathological complete response to therapy. ? Stage: ypT0, ypN0 (sn).

Recommendations:
for adj ttt

MDT Outcome & Plan
PORT
CONTINUE ADJUVANT PHESGO
ADJUVANT AI

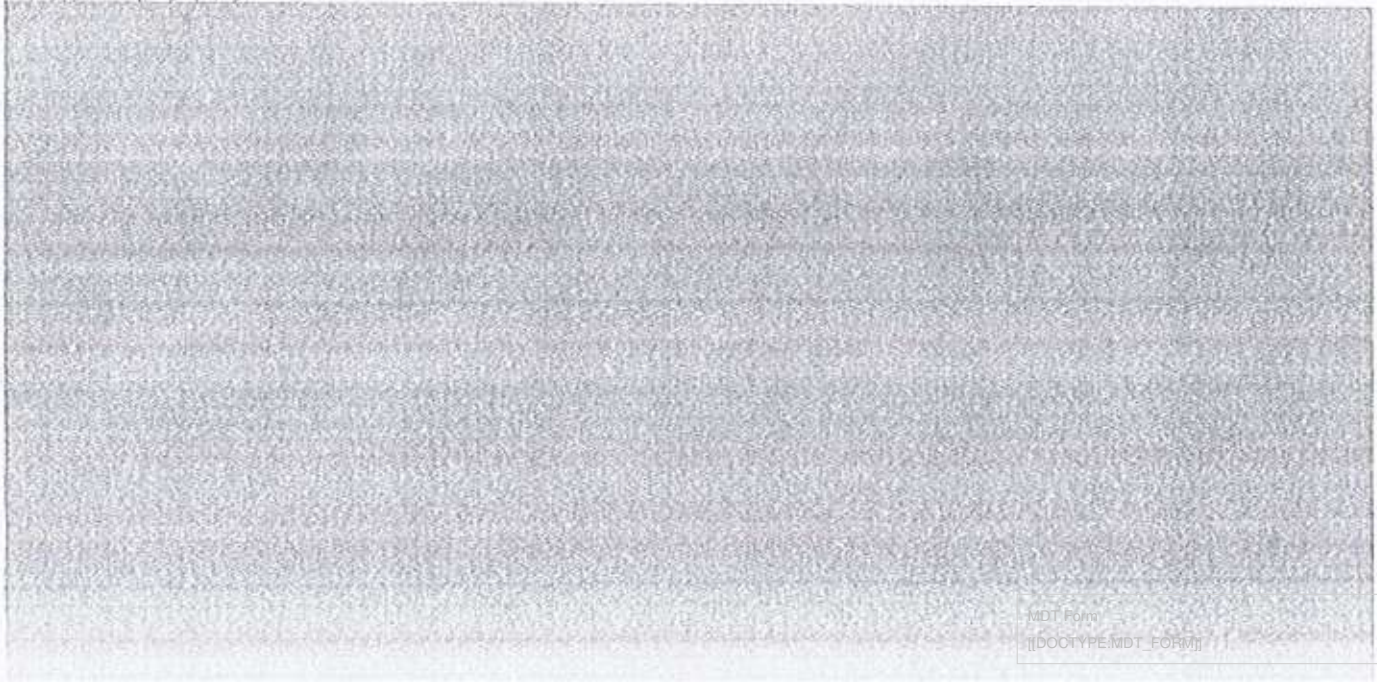
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



MDT Form
[[DOCTYPE:MDT_FORM]]

USTAVE/
OUSSY

Patient Name	:الأستاذه / فاطمه عبدالسلام جوده	Request No.	: 13006324
Patient No.	: 26902172200763	Request Date	: 06/05/2026
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Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY

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Pathology Report

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- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Surgical margins: Free.
* Total nodal status: Eight free lymph nodes (0/8).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

٢١٦٦

Patient ID: 95194	Patient Name English: Fatemah Abdalsalam Godah Hassanen
٥٧ سنة :Age	20260505 10:03:40 AM :Report Date

UltraSound Guided Wire localization:

Clinical summary: Pathologically proven right outer central deeply located breast cancer with clip inserted within and she completed her NACT. For pre-operative wire localization. The lesion shows complete radiological response

Technique:

- Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern in the right breast. One 21G wire was used to localize the lesion.
- The wire is located at 9:00 10.5cm from the nipple.
- The wire is inserted transversely from the outer aspect for about 1.6cm and below the skin for 0.4cm to reach the clip with the wire is transfixing the clip . **confirmed by ML view**

Much obliged

Dr. Marwa Ramzy, MD, FRCR,

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Patient ID: 95194	Patient Name English: Fatemah Abdalsalam Gadah Hassanen
Age: ٥٧ سنة ٢ يوم	2026/05/05 10:03:40 AM :Report Date

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Much obliged

Dr. Marwa Ramzy, MD, FRCR,

Ultrasound
[[DOCTYPE:ULTRASOUND]]

GUSTAVE ROUSSY
INTERNATIONAL HOSPITAL

جوستاف روسي
انترناشونال

ECHO REPORT

NAME: فاطمه عبد الله محمود DATE: 4/5/2026

ID NUMBER: / SEX: ♀ AGE: 58

EF	67 %	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	47	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions — and
LV Wall Thickness — and
LV Diastolic Function — DD grade I
SWMA — SWMA
LA and Ao Root Dimensions — and
Mitral Valve — and
Aortic Valve — and
Tricuspid Valve — Trivial TR
Pulmonary Valve — and
Right Side and Systolic Function — and
Intracardiac Masses and Effusion —

DR.

جوستاف روسي

Patient's Name:	fatma abd el salam gooda
Date:	2026-04-07
Referred by:	Prof.Dr. CT CAP+C
Hospital:	Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

(I) Chest:

- Right breast retro areolar mass is noted measuring 2.6 x 1.6 cm .
- Right enlarged axillary lymph nodes , the largest measures 13 x 6 mm with preserved fatty hilum
- Right paratracheal group lymph nodes , the largest 12 x 9mm.
- Left lower lung lobe area of consolidation intermingled with areas of ground glass attenuation .
- No sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density , no focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Bulky left suprarenal gland .

Echo
[[DOCTYPE:ECHO]]

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name: fatma abd el salam gooda
Date: 2026-04-07
Referred by: Prof.Dr. CT CAP+C
Hospital: Goustave Roussey International Egypt

- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen .
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ Unremarkable other pelvic organs.
- ☐ No retro-crural, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ No worrisome lytic or sclerotic lesions.
- ☐ No vertebral structural collapse .

OPINION:.

- Left lower lung lobe area of consolidation intermingled with areas of ground glass attenuation likely infectious for clinical correlation . .

Dr.Reham Khedr, MD

Dr.Rahma Khader

Dr.Rahma Khader